

Policy on Resources for Infection Prevention and Control

«Hospital Name»

Document control

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Prepared by	Medical Superintendent Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

«Hospital Name» allocates a dedicated budget for infection prevention and control, provides PPE, soaps, and disinfectants at the point of use in every patient-care area, ensures at least one accessible washbasin with running water and hand rub in every patient-care area, and maintains defined isolation and barrier-nursing facilities.

1. Purpose

This policy explains how «Hospital Name» makes available the resources required for infection prevention and control: a dedicated budget, adequate PPE and consumables at the point of use, hand-hygiene facilities in every patient-care area, and isolation and barrier-nursing facilities.

Staffing and competency requirements for the IPC programme are covered in the hospital's other policies.

2. Scope

This policy applies to the Medical Superintendent, the IPC Officer, nursing staff, and all staff responsible for providing or using IPC resources at «Hospital Name».

3. Policy standards

«Hospital Name» allocates a budget line demarcated for IPC activities and ensures required resources — human and material — are available on a continual and sustained basis. PPE, soaps and disinfectants are available at the point of use with adequate inventory maintained;

staff use PPE appropriate to the risk and remove it when the purpose is served. At least one easily accessible washbasin with running water and hand rub are available in every patient-care area. Conditions requiring isolation or barrier nursing are defined in writing; necessary resources and appropriate signage are in place to carry this out.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not run the IPC programme without a budget line demarcated for IPC activities, and do not let required resources go unavailable.
- Do not leave any patient-care area without PPE, soaps and disinfectants at the point of use, and do not allow staff to use PPE inappropriate to the risk or to retain PPE after the purpose is served.
- Do not leave any patient-care area without at least one easily accessible washbasin with running water and hand rub.
- Do not carry out isolation or barrier nursing without the conditions being defined in writing, and do not carry it out without the necessary resources and appropriate signage.

5. What we do

5.1 Make IPC resources continually available

Management ensures that the resources required for the IPC programme — both human resources and materials — are made available on a continual and sustained basis. The annual budget includes a line demarcated for IPC activities, prepared taking into consideration the scope of the programme and previous years' experience.

5.2 Provide PPE, soaps and disinfectants at the point of use

Personal protective equipment — including gloves, protective eyewear, mask, gown, boots/shoe covers, and cap/hair cover — as well as soaps and disinfectants, are available at the point of use. Adequate inventory is maintained. Staff use PPE appropriate to the risks involved and the guidance available, and remove it as soon as the purpose is served.

5.3 Provide hand-hygiene facilities in every patient-care area

At least one easily accessible washbasin with running water is provided in every patient-care area for healthcare providers. Hand rub is also available in every patient-care area — dispensers may be installed at convenient points or carried by staff as they move between patients.

5.4 Provide isolation and barrier-nursing facilities

«Hospital Name» defines the conditions requiring isolation and the conditions requiring barrier nursing (or both). These are carried out when the defined conditions apply. Necessary resources — such as appropriate clothing, masks, gloves, and other PPE — are provided. Appropriate signage is displayed at isolation areas. Patients requiring isolation (contact, droplet or airborne)

are placed in appropriate facilities; airborne isolation facilities are ideally negative-pressure rooms with closed doors.

6. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring IPC resources are budgeted, procured, and available on a continual and sustained basis.
Infection Prevention and Control Officer	Monitors resource availability; escalates shortfalls in PPE, hand-hygiene facilities, or isolation capacity to the Medical Superintendent.
Nursing Superintendent	Ensures PPE, soaps, disinfectants and hand rub are available at the point of use in all patient-care areas and that isolation signage is in place.
Stores / Materials Management	Maintains adequate inventory of PPE, soaps, disinfectants and hand rub; alerts IPC Officer and Nursing Superintendent of low-stock situations.
Quality Coordinator	Audits this policy; holds training records and staff acknowledgements.

7. Quality monitoring

Monitoring area	What is monitored
IPC budget	Annual budget includes a demarcated IPC line; resources available on a continual and sustained basis.
PPE availability	PPE, soaps and disinfectants available at the point of use in all clinical areas; adequate inventory confirmed.
Hand-hygiene facilities	At least one washbasin with running water and hand rub in every patient-care area — verified by IPC round.
Isolation facilities	Conditions for isolation and barrier nursing defined in writing; required resources and appropriate signage in place.

8. Training and staff acknowledgement

All clinical and nursing staff shall be familiar with the PPE types appropriate to different risk situations, the correct sequence for donning and doffing PPE, and the location and use of hand-hygiene facilities and isolation resources.

I have read the Policy on Resources for Infection Prevention and Control of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to the IPC Officer, IPC Nurse(s), nursing staff, stores/materials management, and the Quality Coordinator.

10. Abbreviations

Abbreviation	Meaning
IPC	Infection Prevention and Control
IPCO	Infection Prevention and Control Officer
NABH	National Accreditation Board for Hospitals and Healthcare Providers
PPE	Personal Protective Equipment

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
IPC.2.a	CORE	Sections 3 and 5.1 address the dedicated IPC budget line and the continual and sustained availability of human resources and materials.
IPC.2.b	Commitment	Sections 3 and 5.2 address PPE, soaps and disinfectants at the point of use, adequate inventory, appropriate use by risk, and removal when purpose is served.
IPC.2.c	CORE	Sections 3 and 5.3 address at least one washbasin with running water and hand rub in every patient-care area.
IPC.2.d	Commitment	Sections 3 and 5.4 address written definitions of isolation and barrier-nursing conditions, provision of necessary resources, and appropriate signage.

12. Required Records/Evidence Checklist

IPC budget — IPC.2.a (CORE)

- Annual budget document showing a demarcated IPC line.
- Resource availability records (stock register, PPE log) showing continual and sustained availability.

PPE availability — IPC.2.b

- PPE, soap and disinfectant stock records by area, showing point-of-use availability and adequate inventory.
- IPC round or audit record confirming PPE is available at the point of use.

Hand-hygiene facilities — IPC.2.c (CORE)

- Facilities inventory or IPC round record confirming at least one washbasin with running water and hand rub in every patient-care area.
- Records of any hand-hygiene facility deficiencies and their resolution.

Isolation and barrier-nursing facilities — IPC.2.d

- Written policy or guideline defining conditions for isolation and for barrier nursing.
- Resource inventory showing necessary items (PPE, gowns, signage) are in place for isolation use.
- Audit or round record confirming appropriate signage displayed in isolation areas in use.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. IPC.2.

Disclaimer

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